

I Cancelled My Trip to Florida: When Medicine Becomes a Matter of Belief

I don't know much about Florida, but what I do know is that I won't be getting medical treatment there if I can help it. Not because I do not value the healthcare professionals themselves, but rather because of Senate Bill 1580 (SB1580); filed, approved and enacted in 2023. Signed by Governor Ron DeSantis, SB1580, titled the "Protections of Medical Conscience," claims to shield healthcare providers who object to acts of patient care on conscious grounds, defined as "sincerely held religious, moral, or ethical beliefs (Florida Senate Bill 1580, 2023)." The right to exercise medical conscience is something we haven't been formally introduced to in class – comparatively to autonomy or decisional-capacity. However, "Physician Exercise of Conscience" is a topic that is greatly discussed in the field of bioethics (American Medical Association, n.d.). Primarily seen in conversations around abortion and medical-aid-in-dying, "medical conscience" has received traction in fields where moral disagreement is prevalent. However, SB1580 formally extends this debate to the complete range of medicine.

As previously explored in my essay: "The Over Epidemic: Overcaring, Overprescribing, Overdiagnosing and Overtreatment," there is an ethical framework that medicine abides by – four pillars that dictate the formula for patient care: autonomy, beneficence, non-maleficence and justice. They translate into the doctor's obligation of acting in the patient's best interest, respecting a patient's informed decision, not causing harm to the patient, as well as eradicating any and all discrimination. Conforming to the test of time, the emphasis on each pillar has evolved to accommodate different medical instances and case studies, with its foundations remaining rooted (Roze, 2025). What the practice of healthcare does not account for is personal belief, remaining static in the face of discrimination; moral grounds included. Objectivity is essential to professionalism in medicine while subjective self-declared morals provide bias that interferes with this qualitative care. This is why I propose that the Senate Bill 1580 is an abuse of medical power and should be repealed to reclaim the virtues of care.

Craig Klugman's forum essay on The Hastings Center of Bioethics entitled: "Legalized Medical Discrimination Violates Medical Ethics" highlights the vague nature of the terms used in SB1580,

alongside the blatant legalization of discrimination against patients (2023). Klugman warns of the devastating consequences that could be justified under SB1580, using examples that some have criticized as dramatic and exceptional (2023). However, what is medicine if not dramatic and what is life if not full of exceptions? Of course, there is beneficial intent with the introduction of this bill; including the goal of upholding the Hippocratic Oath (Nair et al., 2024a). It supports Floridians' voices against health policies that do not align with their beliefs, and a physician's right to speak up about such, without the threat of legal repercussions. Yet, to hold healthcare to strict boundaries of the Hippocratic oath would provide an archaic method of treatment (Nair et al., 2024b). To 'do no harm' would conflict with the progression of modern medicine, disregarding the revolutionary intervention of chemotherapy, vaccines, and biopsies, to say the least. Taken literally, even a simple incision would be deemed as a conflict of interest to the Hippocratic Oath (Indla & Radhika, 2019).

Considering SB1580 and its explicit focus on religious, moral, or ethical beliefs, a broader question is posed: should religion be in the same conversation as morality and ethicality? Schuklenk argues that a physician's faith does not belong in the professional exchange between a doctor and a patient (Schuklenk 2019). Doing so could influence the quality of a patient's autonomy, proving religious language and considerations incompatible with the formal duty a doctor is required to deliver (Notini & Oakley, 2023). To be precise, under SB1580 physicians are still expected to provide care in emergencies, honor patients' informed decisions to refuse life-sustaining treatment while also respecting their basic civil liberties. The danger of SB1580 is in its nature to blur the border between personal belief and professional conduct, which can excuse consequences to the vulnerable patient population. Incidents where a practitioner's religion, personal beliefs or morality are noted in the medical decision process can permanently damage the reputation physicians have created with the general public. The imbalance of power between a physician and their patients is clear and consistently under scrutiny, as was made obvious in the Mary Northern case (Northern v. Tennessee Department of Human Services, 1978). SB1580 plays into this imbalance of power, creating a backdoor to misuse this recognition of knowledge and impose on a patient's rights.

To introduce personal elements is inconsistent and can further divide with marginalized communities. A persistent use of SB1580 would only obstruct the ability to treat a patient with the unbiased standard of care they have a right to, subject to moral scrutiny alongside health investigations. Research on the rates of mental health issues among individuals who identify with the LGBTQ+ community quantified the devastation of restrictive healthcare policies. Establishing a link between these policies and significantly higher rates of suicide, suicide attempts, depression, anxiety, and care refusal were found among transgender youth and adults in affected states (Clark, 2024). Cited as the second principle in the World Medical Association's International Code of Medical Ethics: "The physician must practise medicine fairly and justly and provide care based on the patient's health needs without bias or engaging in discriminatory conduct on the basis of age, disease or disability, creed, ethnic origin, gender, nationality, political affiliation, race, culture, sexual orientation, social standing, or any other factor (World Medical Association, 2022)."

Conclusively, I believe the debate surrounding the need for medical consciousness should not be taken into account, and instead be taken out of the picture. While the ideal situation would be repealing SB1580, there is a pressing need to intervene on some behalf through amendment. The urgency present reflects the scope of damage that SB1580 has the ability to justify. There needs to be a requirement of transparency within the conversation between a doctor and a patient in order to balance out the difference in power. Additionally, formal institutions like general Ethics' Boards or the state-specific Medical Association's could disclose notable "conscience" stances that doctors have expressed, in order to ensure the authenticity of their claims. Senate Bill 1580 licenses bias with broad terms, and is in much need of amendment if it is here to stay. Furthermore, having signed this bill Governor DeSantis has reopened the discussion between religion's impact on the practice of medicine, ignoring potential repercussions. A good practice of medicine demands self-restraint and composure, not self-expression. Ultimately, we are left with the question: if a healthcare professional's moral conscience interacts with the integral values of medicine, conflicting to the point of denying a patient the care they are entitled to, is it appropriate for the provider to have the power?

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